Prodigy Anesthesia Quick Review

Bariatrics

What are the three classifications of morbid obesity operations? What operations fall under these classifications?	1. Restrictive (operations include gastric banding and vertical banded gastroplasty) 2. Malabsorptive (jejunoileal bypass) 3. Combination (Roux-en Y gastric bypass) *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 611.*
Open bariatric surgical techniques are organized into two categories. Name and describe each category.	1. Restrictive procedures. These procedures reduce gastric pouch size. The quantity of food that a person can eat at one time, therefore, is limited. 2. Malabsorptive procedures. These procedures produce a chronic malabsorptive state by bypassing the majority of the small bowel. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 7841.*
What general types of open bariatric procedures are the most effective in the promotion of weight loss?	Malabsorptive procedures, however gastric partitioning procedures are more popular because they are associated with fewer complications. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 511.*
1. What is the BMI requirement for a patient desiring bariatric surgery with no associated comorbidities? 2. What is the BMI requirement for patients with associated comorbidities?	1. > 40 kg/m2 2. 35-40 kg/m2 *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 612.*
How should the morbidly obese patient be positioned for preoxygenation?	Reverse trendelenburg position. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 616.*
What are possible postoperative complications associated with open bariatric surgical techniques?	Wound dehiscence, pulmonary embolus, wound infection, leakage of the anastomosis. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 513.*
List possible postoperative complications of laparoscopic bariatric surgery.	Stricture development, anastamotic leak, pneumonia, hemorrhage, thromboembolus, hernia development, obstruction, and infection *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 613.*

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What patient situations would necessitate an open approach to Roux-en Y gastric bypass?	Previous upper abdominal surgery; patients presenting for revision of bariatric surgery; patients who cannot tolerate increased intra-abdominal pressure, such as patients with severe pulmonary disease, CHF, or severe CAD. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 612.*
List benefits associated with a laparoscopic Roux-en Y as opposed to an open approach.	Benefits include: earlier patient discharge, less pain, earlier ambulation, decrease wound complication rates, earlier return to normal activity *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 612.*
A patient presenting for bariatric surgery had preoperative pulmonary function tests performed. What type of breathing pattern would you expect to be reflected in the results?	A restrictive breathing pattern *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 614.*

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